The Alameda tenancy

What they get, what it costs, and what to hold.

Dr John,

This answers the five questions you asked, in the order you asked them, and puts the money on one page you can quote from. The short version is that they take the ground floor except reception and the theatre, the rooms come fitted rather than furnished, and the rate is NGN 470,000 per sqm all in, NGN 53.1 M a year. Quote it as three lines, not one number. That is the difference between a rent they argue about and a cost they can see the basis of.

01 / THE SPACE

The ground floor, less reception and the theatre.

	sqm	What it is
Ground floor, net internal	282.2	Measured from the as-built drawings
Less campus common	(105.2)	Reception, waiting, circulation spine, staircase, vestibule and porch. Serves the whole campus, so it is in the service charge, not the demise
Less the theatre suite	(64.0)	Theatre 32.8, recovery 19.2, dirty utility 6.9, scrub 5.1. Bookable by them, not demised to them
The Alameda demise	**113.0**	**Five to six consulting and procedure rooms, formed from the existing room beside imaging, the open living and dining span and the old lobby**

Say the demise is the ground floor and mean it. It reads as a floor of their own, with its own identity off a shared arrival hall, which is what a medical tourism conversion clinic needs. What it is not is the reception or the theatre, and both of those are worth more to them shared than owned.

02 / WHAT THE RENT BUYS

Fitted and serviced. Not furnished.

	What it covers
Medbury delivers, and recovers in the rate	Partitions, doors and ironmongery. Clinical flooring, welded and coved. Hand-wash to every clinical room. Cooling and mechanical fresh air. Power, lighting and emergency lighting. Data and network. Fire detection and alarm. Room signage. Decoration
Alameda brings	Clinical equipment and instruments. Consumables. Medical furniture specific to their protocols. Their own devices and endpoints. Their own consulting room furniture if they want a particular look
Neither, until asked	Desks, chairs and soft furnishing to a hotel standard. If they want the rooms furnished as well as fitted, that is a separate quote and a separate line. Do not fold it into the rate

The distinction matters in the room. **Fitted** means the rooms are clinically usable on handover: walls, floors, water, power, air and light to a standard that will register. **Furnished** means somebody chose the chairs. We are offering the first and pricing the second separately, and the fit-out recovery line in section 05 is how the first is paid for.

03 / ACCESS AND PERIOD

Five years, and their own front door hours.

Term	Five years	Long enough to recover the fit-out over the same period, which is what makes the recovery line fair rather than a surcharge
Deposit	6 months	Rent and service charge, held and applied at the end
Campus window	07:00 to 21:00, Monday to Saturday	When reception, security, cooling and the clinical support services run
Their access	Unrestricted within the window	It is their demise. They set their own clinic hours inside it and do not book time
Outside the window	By arrangement	Sundays and late lists can be run, and carry the marginal cost of opening the building. Agree the rate now rather than the first time they ask
Patient arrival	Through the single campus reception	There is one patient entrance. Their patients are received, identified and directed by our front office, then seen by their clinicians

04 / SHARED AND THEIRS

The line to draw, and where.

	What sits there
Theirs alone	The 113.0 sqm demise. Their clinical staff, their patient relationship, their records and clinical governance, their branding within the demise, their protocols and their fees
Shared, and in the service charge	Reception, waiting and concierge. Circulation and the staircase. Patient WCs. Security, cleaning, waste and grounds. Power, cooling and water. Fire systems. Network and connectivity. Facilities management
Shared, and charged on use	The day case theatre, by the list. Digital X-ray and phlebotomy. The laboratory in the chalet. The pharmacy collection point. Sterilising
Ours alone	Campus branding and the building identity. The first floor. The reception function itself. Anything that touches another occupier

The principle to state, because it answers most of their questions before they ask. Anything that serves only them sits in the demise and they control it. Anything that serves the campus sits with us and reaches them through the service charge. Anything they consume by the unit is charged by the unit. Three categories, and every request they make falls into one of them.

05 / THE MONEY

One rate, quoted as three lines.

	NGN / sqm / yr	NGN M / yr	What it is
Rent	128,971	14.6	The space itself, at a landlord margin over the head rent
Fit-out recovery	82,403	9.3	Their share of the clinical fit-out, recovered across the five year term. It falls away at renewal
Service charge	258,626	29.2	Their share of power, security, cleaning, facilities, insurance, regulatory, reception and ICT
Total	**470,000**	**53.1**	**NGN 4.43 M a month. About USD 34,265 a year**

The number not to go below. Housing them costs the SPV NGN 46.0 M a year once the head rent, the fit-out and a fair share of campus services are counted. The quote is NGN 53.1 M, so the margin is NGN 7.1 M. **Below NGN 46.0 M the campus is housing a tenant at a loss, and Alameda captures 51% of that loss through the joint venture.**

06 / THE 49 PER CENT

Why the rate has to be billed properly, not softened.

The tenant is not Alameda. It is the joint venture between Medbury and Alameda, and **Medbury owns 49% of it**. That is worth understanding before the meeting, because the instinct it creates is the wrong one.

The instinct is that since Medbury pays 49 kobo of every naira it charges, the rent matters less and can be softened to buy goodwill. **The opposite is true.**

Discount NGN 1 M off the rate, and	NGN M
Medbury loses, in rent	(1.00)
The clinic's profit rises by	1.00
Medbury takes back its 49% of that	0.49
Net to Medbury	(0.51)
Straight to Alameda	**0.51**

Every naira off the rate is 51 kobo handed to the majority partner. A NGN 10 M concession is not a NGN 10 M gesture, it is a NGN 5.1 M transfer to Alameda and a NGN 5.1 M cost to Medbury. Softening the rent is the single most expensive way to be generous in this negotiation.

There are two further reasons the rate has to be real. The joint venture agreement requires related party transactions at arm's length, and Nigerian transfer pricing rules mean FIRS can adjust a below-market related-party rent and disallow the deduction. And the clinic's own accounts have to be honest: if it is housed below cost it looks profitable when it is not, which misleads both shareholders and makes the second site impossible to price.

Bill it at NGN 53.1 M, and hold it. Housing them costs NGN 37.5 M, so the margin is NGN 7.7 M. At NGN 37.5 M the margin is nil. Below that Medbury is funding a subsidy of which Alameda captures 51%. The 49% is a reason to bill properly, not a reason to bill less.

Press for first call on diagnostics, pharmacy and theatre as well, and press hard. Those return whole with no 49% deduction. But take them **in addition to** a properly billed rate, never instead of one.

07 / THE SERVICE CHARGE

The line that was wrong, and the base that fixes it.

The campus model apportioned the service charge across **net internal area**. That is the wrong base. Service charge is apportioned across **occupied** area, because the common parts are what the charge pays for and they cannot pay their own bill.

Apportionment base	Their share	Service charge	Cost to the SPV
Net internal, as modelled	17.0%	16.8	37.5
All occupied space	25.6%	25.3	46.0
Lettable only	31.4%	31.1	51.8

Use occupied space, the middle row. Medbury's own clinics upstairs, the theatre, diagnostics and the pharmacy all use the same front office and the same generator. Leaving them out of the base makes the outside tenant pay for them, which is not defensible if Alameda ever asks how the number is built. Lettable-only is the other extreme and loads everything onto three demises.

On that base the service charge is NGN 224,134 per sqm rather than NGN 149,029, and the cost of housing the clinic is **NGN 46.0 M**, not NGN 37.5 M. The old NGN 400,000 quote was 98% of cost. It had no margin in it at all.

Diesel, and why no separate deposit is needed

Power is the largest thing in the service charge and the most volatile. Three terms handle it without asking Medbury to carry diesel for five years.

Term	What it does
Monthly in advance	They pay in the month for power bought in the month. This removes the float, which is the real exposure. A deposit does not: it sits there, it does not buy next month's tanker
Reconciled annually	Billed on account against audited campus cost, with a balancing charge or credit. Diesel volatility passes through instead of being absorbed
Sub-meter the demise	Their own rooms metered and recharged at cost, or a prepaid meter if they prefer. NGN 0.3 to 0.6 M to install against a NGN 10 M a year exposure

A separate diesel deposit would be charging twice. The six month deposit already holds NGN 13.3 M against service charge, which is more than a year of their power share on any of the scenarios. Ask for advance payment and reconciliation instead.

08 / WHAT ELSE THEY GET

The reason the rate is what it is.

	What it is
Diagnostics on site	Laboratory in the chalet, digital X-ray and phlebotomy on the ground floor beside them, a draw point upstairs. Their patient is worked up in the building rather than sent across Abuja
A day case theatre	On their own floor, level access, bookable by the list. Nothing else they could rent in Abuja at this size comes with one
Pharmacy	Dispensary and cold chain in the boys' quarters, collection hatch in the waiting hall. Their script is filled before the patient leaves
A front office they do not staff	Reception, patient identification, navigation and the arrival experience, run to one standard across the campus
Referral flow	From the family practice and the sessional panel upstairs. Not promised as a volume, but it is real and it is why a medical tourism conversion clinic sits better here than in a standalone unit
Credentialing support	Temporary registration and immigration administration for visiting clinicians, which is process they would otherwise carry themselves
Revenue cycle management	Billing and collection, if they want it. Priced separately and worth offering, because collection is where most visiting-clinician models leak

09 / FOR THE ROOM

Four things to hold.

1	Quote three lines, never one	A single number invites a haggle. Three lines invite a conversation about what each one buys, and every one of them is defensible from cost
2	The fit-out recovery is the renewal argument	It falls away after five years. Say so. It gives them a reason to renew and gives us a rate rise that costs them nothing
3	Furnishing is a separate quote	If they want the rooms furnished, price it separately. Folding it into the rate hides it and we will never get it back
4	Service charge monthly in advance, reconciled annually	The one term that protects the whole five years from diesel. Ask for it as normal practice, because it is, and it will not be resisted
5	Out of hours access needs a rate today	Sundays and late lists will be asked for. Agree the basis now, while it is hypothetical and cheap, rather than the first time it is urgent

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Areas are measured from the as-built drawings and recover the CAD to about 10 mm. The rate is built up from cost rather than benchmarked, and the build-up is in scripts/lyfeplace_campus.py. Chalet and boys' quarters areas remain estimates because there is no as-built drawing for

either in the pack.